A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr Basit Khan
Shepparton GP Medical Practice And Acupuncture Clinic

Dear Dr Khan,

Re: Mrs Kylie Dennis
DOB: 26/08/1973 Phone: 0435 624 380
Address: 162 Racecourse Road, BENALLA, VIC, 3672
Email: kylie_tanya@hotmail.com
Medicare: 2404698815 / 7

Summary

- **Ongoing bloating and abdominal distension related to visceral hypersensitivity.**
- **Trial of low-dose nortriptyline with upward titration to 30mg.**
- **Secondary plan for a two-week course of rifaximin if symptoms persist.**
- **Management of concurrent constipation with stool softeners.**
- **No action required by you.**

I had the pleasure of conducting a review with Kylie Dennis regarding her ongoing gastrointestinal symptoms. She continues to experience significant bloating and abdominal distension, noting that her symptoms are frequently triggered by dietary factors, specifically garlic and onion. While she had previously lost weight, this has now stabilised, and she has regained some weight following recent injuries that have limited her activity levels.

Kylie's bowel habit remains suboptimal, characterised by hard and infrequent stools. We discussed her current medication regimen, which includes desvenlafaxine. I have cautioned her regarding the potential risk of serotonin syndrome if nortriptyline is combined with desvenlafaxine or other agents such as tramadol or phentermine. However, I expect the risk to be minimal at the conservative doses we are utilising.

Impression and Plan

My clinical impression is that Kylie’s symptoms are predominantly driven by visceral hypersensitivity. To address this, I have initiated a trial of nortriptyline. I have instructed her to commence at 10mg nocte and slowly titrate the dose up to a maximum of 30mg, depending on her tolerance and symptomatic response. I have warned her of potential side effects, including xerostomia and morning somnolence.

Because nortriptyline can exacerbate constipation, I have advised Kylie to use stool softeners such as docusate sodium or macrogol. I have specifically counselled her against the regular use of stimulant laxatives like senna to avoid bowel reliance.

If the nortriptyline trial does not provide adequate relief after approximately eight weeks, the second-line strategy will be a two-week course of rifaximin to modulate the gut microbiota. I have explained that this medication is not covered by the PBS for this indication and advised her to seek a compounding pharmacy to

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manage the cost. I have provided Kylie with scripts for both medications to allow for a sequential trial. I intend to review her progress in approximately four months.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: A/Prof Michael Hii

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065